

العمر (تاريخ الميلاد) : 18-01-1967 المهنة : ربه منزل

التوقيع: (-----)

الذين يتلقوا علاج هرموني حمض الزيلودورونيك [14157]

----- التعليق على التشخيص :

تتعمد صحة الأبحاث المرفوعة  
رئيس اللجنة ومدير  
المستشفى.

رئيس  
المجلس الطبي المتخصص

Name: جميله عبدالرازق عبدالوهاب الشامى  
National ID: 26701181702482



## تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :  
PM 01:17 05-07-2026

تاريخ ومساءة الدخول :  
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :  
د/شيماء لاشين

جمهورية مصر العربية  
بشارة حبيب الدجوي

جسده  
عبدالرزاق عبدالقادر الشامي  
٢٥ شب - كاتبة الاحكام العاصي  
الطار - القاهرة

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يونس صلي

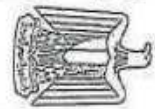
مترجمة

مسلمة

إثني

احمد داود السيد داود

البطاقة سارية حتى ٢٠٢٧/٢/٠٩



|                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            |                       |                                                                                     |
|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------|-------------------------------------------------------------------------------------|
| Name: <b>جميله عبدالرازق عبدالوهاب الشامى</b>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              |                       |  |
| MR. : <b>8369</b>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |                       |                                                                                     |
| Detailed Medical Report                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    |                       |                                                                                     |
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| تحتية طبية وبعد,,                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |                       |                                                                                     |
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|                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            | رقم طبي:              | بقسم:                                                                               |
| ننشرف بأن نوضح فيما يلي تقريراً عن حالته                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   |                       |                                                                                     |
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| وبعد الكشف والفحوصات والمتابعة وجد انه                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     |                       |                                                                                     |
| <p>The patient has a history of post-mastectomy breast cancer treated with right modified radical mastectomy in April 2018 (pathology T2 N1). The tumor was estrogen-receptor positive, progesterone-receptor positive, HER2-positive, and of the invasive ductal carcinoma (IDC) histology. Following surgery, the patient received adjuvant hormonal therapy with Femara from September 2018 until November 2023. In January 2024, metastatic disease to bone was identified, and systemic therapy with Faslodex (fulvestrant) and Ibrance (palbociclib) was initiated. A bone scan performed on 4 May 2026 demonstrated two osseous lesions involving the cervical vertebrae and right sacroiliac joint. Physical examination revealed right upper limb weakness. The plan includes palliative radiotherapy to the cervical vertebrae, continuation of Faslodex and Ibrance, and administration of Zometa (zoledronic acid). Current status: awaiting palliative radiotherapy; systemic therapy ongoing.</p> <p>The patient has not yet received the planned palliative radiotherapy for the cervical vertebral metastases; systemic therapy with Faslodex and Ibrance continues, and recent laboratory results have been reviewed and accepted. Current status: pending radiotherapy; labs reviewed.</p> <p>The patient continues on the current regimen of Ibrance combined with Faslodex 500 mg. Imaging studies, including a CT of the chest, abdomen, and pelvis and a repeat bone scan, have been ordered. The next follow-up visit is scheduled for 28 September 2026. Current status: ongoing therapy with scheduled imaging and follow-up.</p> |                       |                                                                                     |
| تحريرا في ,                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                |                       |                                                                                     |
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|                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            | dina makram           | الطبيب المعالج :                                                                    |
| يعتمد ,                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    |                       |                                                                                     |

**Mrs. GAMILA ABD EL RAZIK ABD EL WAHAB**  
**Date. 04/05/2026**

**TC-99m MDP WHOLE BODY BONE SCAN**

**History**

- 53-year-old lady who gave history of breast cancer and referred for follow up.

**Technique**

- Anterior and posterior whole-body scans were acquired 2 hours post injection of the tracer.

**Findings**

**In comparison to the last bone scan in 29/12/2025.**

- The current study revealed regressive course of the previously detected osseous lesions at the cervical spine and right sacroiliac region.
- No newly developed osseous lesions could be detected.
- Degenerative changes are noticed at the spine and big joints.
- The dense tracer uptake seen at the right clavicular region is attributed to port-catheter theine.

**IMPRESSION**

- Two osseous lesions as described above, with regressive course.
- Follow up is recommended to assess response to therapy.

**Much obliged,  
Dr. Ahmed NASR, MD  
Nuclear Medicine Consultant**



**Name: Gamela Abd Al Razak Abd Al Wahab**

**Date: 8/5/2026**

**Prof. Dr.:**

**Examination:**

- **Multi-slice Computed tomography of the Chest.**

**Technical data:**

- Contiguous axial 5 mm CT scans of the chest following intravenous contrast injection with viewing sets in the high resolution lung window as well as mediastinal window.
- Coronal MPR.

**Findings:**

- Normal volumes of both lung fields which are normally aerated. No evidence of focal or active parenchymal lesions.
- Normal appearance of the tracheobronchial tree.
- No evidence of hilar or mediastinal mass lesions or lymphadenopathy.
- No evidence of pleural collections or thickening.
- Normal cardiac size and shape, with no pericardial effusion.
- Normal size and shape of the aorta and pulmonary trunk.
- Scans through the upper abdomen are unremarkable.

**Conclusion:**

- Unremarkable CT examination of the chest.

**Dr/ Mohamed Eid MD**

## MDT FORM

9/12/2025 G

|                   |                |                |                 |
|-------------------|----------------|----------------|-----------------|
| Name              | (Sajid Ali)    | MRN            | 243/2018        |
| Age               | 58             | ID             | CN-11A-IV-CEAK  |
| Menopausal status | Postmenopausal | Phone          | 0115162173      |
| NRS SCORE         | P.S. 1         | Marital status |                 |
| BMI               |                | Family history |                 |
| CO morbidity      | HTN            | REASON OF MDT  | HT Continuation |

## Clinical Data

BX: IDC

R: T4 N1/3 M0

ER: +ve PR: +ve HER2: 2+ side not exp Ki67: +ve

## Sonomammography:

2018/1/1 Underwent R MRM T2 N1 → AC → PET

## Mets Workup:

pancrea till 2023  
Bone mets TNM

Decis for Hormone + Fulvestrant  
• If is on (ibuprofen/Fulvestrant) for bone mets ⇒

## Recommendations:

till No last Bone Scan 5/2025 → SD  
• CT-CAP 10/2025 → free

If is off for 3 m now & needs  
MDT app. to resume

## MDT OUTCOME &amp; PLAN

- ① Paclitaxel + Fulvestrant.
- ② Zometa/3 m

## SIGNATURE

| MEDICAL ONCOLOGY | SURGERY | RADIOLOGY   | PATHOLOGY | Nutrition | Psychiatric |
|------------------|---------|-------------|-----------|-----------|-------------|
| (Signature)      |         | (Signature) |           |           |             |

|                 |                                      |                    |                                   |
|-----------------|--------------------------------------|--------------------|-----------------------------------|
| Patient Name    | : Mrs. / جميلة عبد الرزاق عبد الوهاب | Request No.        | : 13001228                        |
| Patient No.     | : 243-2018                           | Request Date       | : 14/12/2023                      |
| Path No.        | : SR-1586/23                         | Verification Date  | : 14/12/2023                      |
| Sex             | : Female                             | Financial Category | : أرقام دالر السلام - تلفة الدولة |
| Age             | : 52 Y                               | Department         | : Out Patient                     |
| Referring Doc : |                                      | Registered by      | : marwa mohamed youss             |

### Pathology Report

Clinical data  
Breast mass

Nature of specimen  
Revision of slides  
Gross examination  
One block ( 150218) & 1slide were received

Microscopic examination  
\* Histological type: Invasive duct carcinoma  
\* Grade ( differentiation +nuclear pleomorphism+mitosis ): (3+2+2) Grade II  
\* DCIS: No  
\* Angiolinvasion: Could not be detected within the limits of biopsy received  
IPT :

ER: Strong nuclear staining in 95% of tumor cells (8/8)  
PR: Moderate nuclear staining in 5% of tumor cells (4/8)  
Her2: faint incomplete membrane staining in >10 of tumor cell , score( +1)  
Ki 67: Positive nuclear staining in 20% of tumor cells

Diagnosis  
Breast mass ; slide revision :  
Invasive duct carcinoma, Grade II .  
IPT :

ER: Positive (8/8)  
PR: Positive (4/8)  
Her2: Negative score( +1)  
Ki67: Positive in 20 % of tumor cells

Comment: Although Her2 is Negative, Such cases (Score 1+) are predictive of response to Her2-Ab drug conjugates (ADCS) according to latest 2023 ASCO-CAP guidelines.

Verified by :  
Dr. Heba Hasan

Printing Date : 02/11/2024 14:20:46

Page 1 of 2

Printing By : Dr. Heba Hasan



طرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مبنى الجامعة - القاهرة - مصر  
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Mir El Qadenia - Cairo, Egypt

|                |                                    |            |     |
|----------------|------------------------------------|------------|-----|
| Patient Name:  | GAMELAH ABDELRAZEK                 |            |     |
| Patient Id:    | 185                                | Sex:       | F   |
| Study:         | CHESTABDPLV+C                      | Age:       | 58Y |
| Date:          | 20/10/2025                         | Accession: |     |
| Dear Prof. Dr. | Thank you for referring this case, |            |     |

### **NON-CONTRAST ENHANCED SPIRAL MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS:**

- *Post right radical mastectomy with clear operative bed is seen.*
- *The scanned lung fields show no focal metastatic deposits seen.*
- *The liver is average in size and shows no focal lesions depicted.*
- *No IHBR dilatation.*
- *The Gall bladder is distended and shows multiple large calcified stones.*
- *The body and tail of the pancreas are normal. No masses seen.*
- *No para aortic or peripancreatic masses or lymphadenopathy seen.*
- *No free ascites.*
- *No peripancreatic collections or dilated ducts.*
- *The liver is average in size with homogenous hypoattenuated CT texture denoting mild fatty infiltration. No focal lesions. No intra-hepatic or extra-hepatic biliary radicles dilatation.*
- *Normal size & parenchymal texture of the spleen with no focal lesions seen. No radio-opaque calculi or pathological calcification seen within both kidneys, both ureters or the urinary bladder. There is dense calcified right pelvic lymph node and phlebolith.*
- *Normal size and parenchymal thickness of both kidneys with no back-pressure changes. No solid or cystic lesions depicted.*
- *Right ovary is normal.*
- *Normal AVF uterus and normal CT attenuation of its myometrium and cavity.*
- *Clear scanned lung bases.*

#### **Opinion:**

- *The hall mark of the study is the presence of a well defined left*

|                |                                    |            |     |
|----------------|------------------------------------|------------|-----|
| Patient Name:  | GAMELAH ABDELRAZEK                 |            |     |
| Patient Id:    | 185                                | Sex:       | F   |
| Study:         | CHESTABDPLV+C                      | Age:       | 58Y |
| Date:          | 20/10/2025                         | Accession: |     |
| Dear Prof. Dr. | Thank you for referring this case, |            |     |

***ovarian mature follicular cyst 33 mm in diameter and shows well defined enhancing wall, no mural nodules. There is mild free pelvic fluid collection representing mid cyclic fluid colle Post right radical mastectomy with clear operative bed is seen.***

- ***The scanned lung fields show no focal metastatic deposits seen.***
- ***Otherwise, normal CT scan of the abdomen and pelvis.***

***Dr / Waseem El Gendy, MD***

|                                                         |                   |
|---------------------------------------------------------|-------------------|
| Patient Name: Gamila Abd Al Razek Abd Al Wahab Al Shamy | Date : 03/10/2024 |
| Exam : C.T CHEST                                        | Age : 57 Year(s)  |
| Medical ID: 243/2018                                    | Pat. ID: 41719    |

**NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS:**

Compared to previous study dated 16/05/2024.

**Chest:**

- Status post right mastectomy showing operative bed masses or collection.
- **Stable** few scattered bilateral pulmonary nodules, the largest is seen at the apical segment of the right upper lung lobe measures about 4 mm.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Right sided Port-A-Cath is still noted.

**Abdomen & pelvis:**

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities on non-contrast basis
- Still noted calculi gall bladder with mild circumferential mural thickening.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas.
- Still noted calcification of both suprarenal glands, likely old granulomatous infection.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- **Stable** pancreatic uncinata process cystic lesion still measures about 3 cm.
- No definite worrisome lytic or sclerotic osseous lesions.

**Opinion:**

1. Status post right MRM showing no sizable hepatic deposits on non contrast basis.
2. **Rather stationary** few small bilateral pulmonary nodules.

Dr. Ghada H. Khalifa, MSc

**TO: THE HONORABLE CHIEF OF THE HONORABLE**

**History**

- 56-year-old lady, presented with breast cancer and referred for status evaluation.

**Technique**

- Anterior and posterior whole-body scans were acquired ٢ hours post injection of the tracer.

**Findings**

- In comparison to the last scan dated ١١/١١/٢٠٢٤, today study revealed stationary course of two active foci which are located at cervical vertebra, and right sacroiliac articulation.
- No newly developed osseus lesions could be detected.
- Degenerative changes are noticed at the rest of spine and big joints.

**IMPRESSION**

- Two osseous lesions as described above with stationary course.
- Follow up bone scanning is recommended to assess the response to therapy.

Much obliged,  
Dr. Ahmed NASR, MD  
Nuclear Medicine Consultant

### TC-99m MDP WHOLE BODY BONE SCAN

#### History

- 55-year-old lady, presented with left breast cancer and referred for status evaluation.

#### Technique

- Anterior and posterior whole-body scans were acquired 2 hours post injection of the tracer.

#### Findings

- In comparison to the scan in 01/10/2023, the current scanned skeleton revealed rather stationary course of the multiple active foci.
- They involve cervical spine, right sternoclavicular articulation, and right sacroiliac joint.
- No newly developed osseous lesions could be detected.
- Degenerative changes are noticed at the rest of spine and big joints.

#### IMPRESSION

- Abnormal osseous lesions as described above with stationary course.
- Follow up bone scanning is recommended to assess the response to therapy.

Much obliged,  
Dr. Ahmed NASR, MD  
Nuclear Medicine Consultant

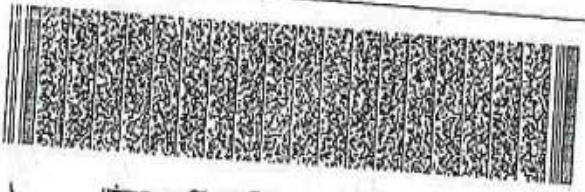
جمهورية مصر العربية  
بطاقة تحقيق الشخصية



حميله  
عبدالرازق عبدالله هاني الشامي  
٥ ش - كنيسة الاتحاد السكائيتي  
الظاهر - القاهرة

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الطاقة سارية حتى ٩

احمد داود السيد داود

التي

مستلمة

مروجة

بدون صل





|                                                |                  |
|------------------------------------------------|------------------|
| Pat. Name: Gamila Abd Al Razeq<br>Abd Al Wahab | Pat. ID: 3642396 |
| Procedure Name: Bone Scan                      | Date: 14/09/2026 |
|                                                |                  |

### Tc-99m MDP Bone Scan

Compared to the last bone scan report only dated 4/5/2026 , the current study revealed :

NO currently detected active or cold lesions at the right sacroiliac region.

Still seen non uniform tracer uptake at the cervical region with active uptake at the lower cervical vertebrae more evident on the anterior view , for further correlation with CT cuts.

Rather linear active uptake is still seen opposite the right sterno-clavicular region of equivocal nature , ?? representing right parasternal lesion for further correlation with CT cuts.

Currently detected active lesion is seen at the trochanteric area of left femur, ?? newly developed metastatic deposit for further assessment by CT cuts.

Degenerative changes of the spine and arthritic changes of both shoulders and knees are observed.

### OPINION:

Active osseous lesions at the right upper parasternal region , lower cervical vertebrae and left femoral trochanteric region.  
Bone metastases therein are suggested for further correlation with CT cuts.

Thanks For reference  
Nuclear medicine Unit  
DR. AHMED ZAHER , M.D.  
Prof. Of Nuclear Medicine , NCI ,  
Cairo University

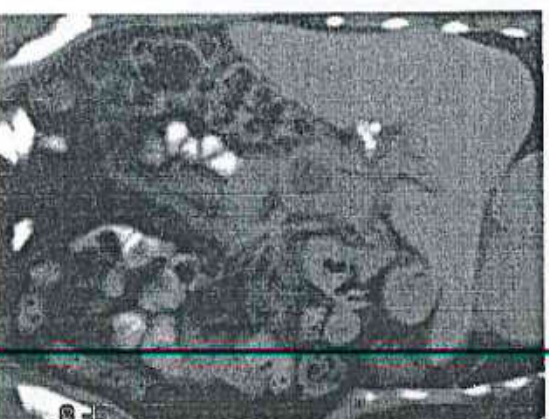
For any inquiries regarding the findings in the report please contact us on  
[radreport@cairoscan.com.eg](mailto:radreport@cairoscan.com.eg)

|                |                                    |            |     |
|----------------|------------------------------------|------------|-----|
| Patient Name:  | GAMELA ABDALRAZEQ A.ALWHAB         |            |     |
| Patient Id:    | C A P+C                            | Sex:       | F   |
| Study:         | Abdomen^ABDOMED (Adult)            | Age:       | 54Y |
| Date:          | 07/09/2026                         | Accession: |     |
| Dear Prof. Dr. | Thank you for referring this case, |            |     |

**POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN & PELVIS**  
**REVEALED:-**

**I-CT CHEST FINDINGS:**

- Right MRM is noted.
- Right sided porta cath is seen..
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No CT evidence of enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.
- Normal appearance of the dorsal vertebrae.
- Upper abdominal cuts are unremarkable.



**OPINION:**

- *Essentially normal CT study of the chest.*

|                |                                    |            |     |
|----------------|------------------------------------|------------|-----|
| Patient Name:  | GAMELA ABDALRAZEQ A.ALWHAB         |            |     |
| Patient Id:    | C A P+C                            | Sex:       | F   |
| Study:         | Abdomen^ABDOMED (Adult)            | Age:       | 54Y |
| Date:          | 07/09/2026                         | Accession: |     |
| Dear Prof. Dr. | Thank you for referring this case, |            |     |

## II-CT ABDOMEN & PELVIS FINDINGS:

The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.

- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicle dilatation.
- Variable sized gall bladder stones are noted.
- Well defined cystic lesion realted 2<sup>nd</sup> part of dudenoum measures ( 3x2.8 cm ) .
- No abnormal contast enhancement is noted.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra renals.
- No abdominal or pelvic lymph node enlargement seen.
- No ascites nor abdominal collections noted.
- Normal appearance of the urinary bladder with no intra-vesical masses, filling defects or diverticular outpouching.
- Normal CT appearance of the other pelvic organs.
- Clear scanned lung bases.

## Opinion:

*Past history of right breast cancer , underwent MRM , CTH , in correlation with pervious studies dated 08.05.2026, current study reveled:*

- *No evidence of abnormal masses or enhancement.*
- *Calcular gall bladder.*
- *Well defined cystic lesion related to 2<sup>nd</sup> part of dudenoum likely diverticulum.*

*Dr. Waseem El Gendy, MD.*



د / وليد عبد المادني  
استاذ م . بكلية الطب - جامعة عين شمس  
دكتوراة الباثولوجيا الكلينيكية ( أمراض العظام)

معامل  
الاعتماد

تاريخ العينة : PM 12:08 - 2026/09/28

تاريخ التسليم : PM 02:23 - 2026/09/28

رقم الملف : 194109-1

58 Y : السن

Female : النوع

الاسم : جيهله عبدالرازق  
رقم الزبارة : 109260972  
القسم :  
الطبيب :



### Chemistry Report

| Test                  | Result | Unit  | Ref. Range     |
|-----------------------|--------|-------|----------------|
| Serum Urea            | 42     | mg/dl | ( 15 - 50 )    |
| Serum Uric Acid       | 5.7    | mg/dl | ( 2.4 - 6.0 )  |
| Serum Creatinine      | 0.9    | mg/dl | ( 0.6 - 1.1 )  |
| Serum Calcium (Total) | 9.8    | mg/dl | ( 8.4 - 10.2 ) |
| BUN                   | 20     | mg/dl | ( 7 - 23 )     |

Signature

د. وليد المادني



العليسية : ٣٥٩ شارع رمسيس ادم مستشفى العمراني باب الجردة ت : ٠١٠٤٠٧٢٦٦١٢ - ٢٦٨٣٩٤٠٠  
مصر الجديدة : ١ شارع المنزه بجوار مستشفى هليوبوليس ت : ٠١٠٤٠٥٩٦٠٠٥ - ٢٠٦٧٢١٤٤  
مدينة العوير : سنتر الجازل - الحي الاول ت : ٠١٠٤٠٧٢٦٦١٣ - ٤٤٩١٦٦٣٦  
٠١٢٠٣٢٦٧٣٦٦ - ٠١١٠٢٨٠٣٩٥٥ - ٠١٠٤٠٧٢٦٦١٢ - ٢٦٨٣٩٤٠٠  
٠١١٠٢٨٠٣٩٤٤ - ٠١٢٧٤٩٧٦٨٠١ - ٠١٠٤٠٥٩٦٠٠٥ - ٢٠٦٧٢١٤٤  
٠١٢١٢٣٤٩١٦٢ - ٠١١٠٢٨٠٣٩٤٣ - ٠١٠٤٠٧٢٦٦١٣ - ٤٤٩١٦٦٣٦